

Figure 1. Cohort assembly and analysis populations

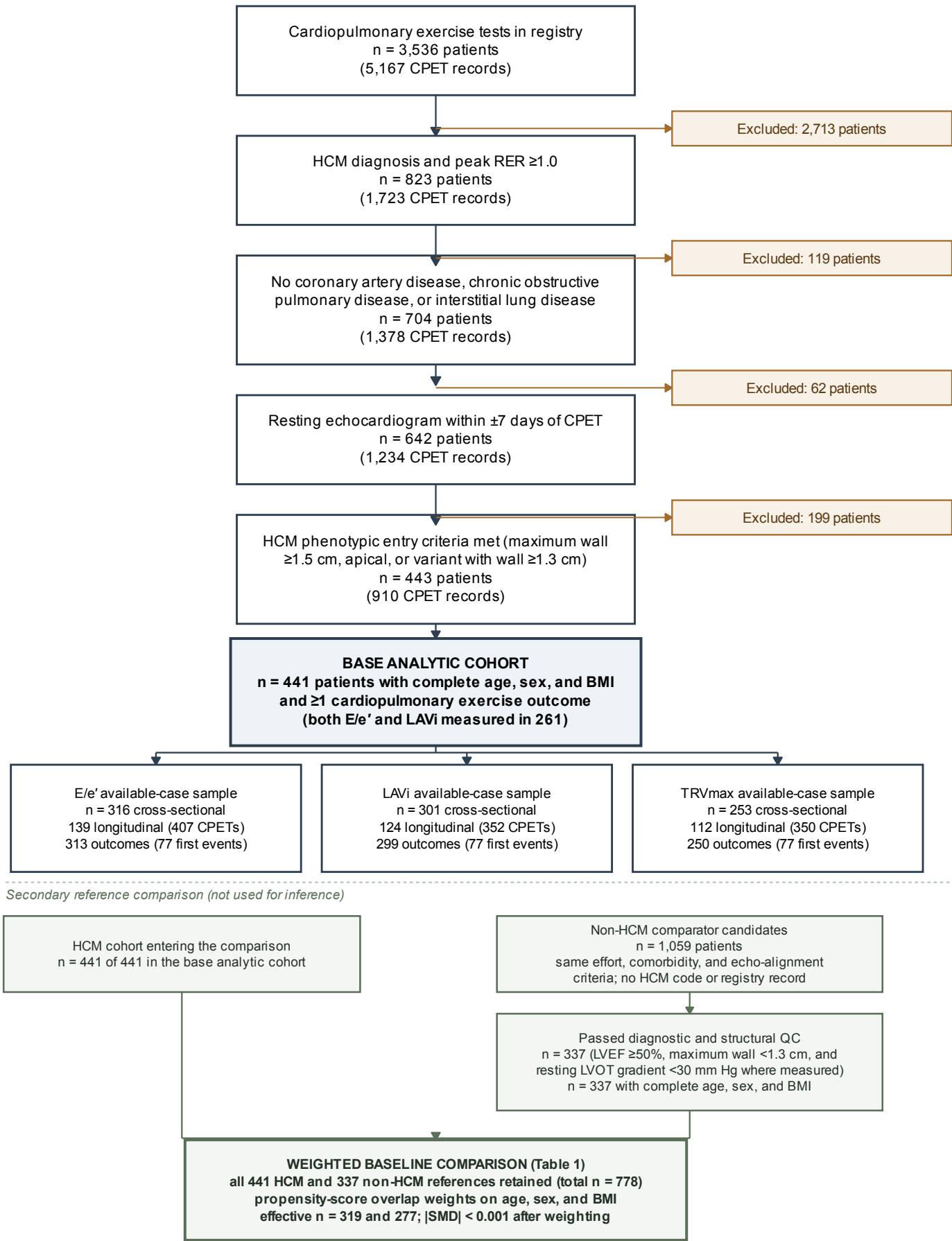


Figure 1. Cohort assembly and analysis populations. The vertical spine reports sequential HCM cohort entry, with exclusions at each step shown at right. Patients meeting phenotypic entry criteria with complete age, sex, and body mass index and at least one cardiopulmonary exercise outcome form the base analytic cohort. Each resting LV diastolic parameter is then analyzed in every patient in whom that parameter was measured, so the three branches carry their own cross-sectional, longitudinal, and clinical-outcome denominators; requiring two or three parameters simultaneously would remove patients from the analysis of the parameter that was measured least often. The subset with both E/e' and LAVi measured is retained as a prespecified consistency check. The secondary baseline comparison joins the HCM cohort to phenotypically screened non-HCM comparator candidates and supports Table 1 only. Every eligible patient is retained and comparability is achieved by propensity-score overlap weighting on age, sex, and body mass index rather than by discarding unmatched patients; absolute standardized differences are below 0.001 after weighting (0.92 for age before), and effective sample sizes are reported in the box. One-to-one caliper matching of the same pools is reported as a sensitivity analysis. All counts are computed by the analysis pipeline at render time. CPET = cardiopulmonary exercise testing; LAVi = left atrial volume index; LVEF = left ventricular ejection fraction; LVOT = left ventricular outflow tract; QC = quality control; RER = respiratory exchange ratio; TRVmax = maximum tricuspid regurgitant velocity.